

Dr. Ian Robson

Allergist

Central Hospital

Oldtown

15th day - 1st visit IL 1, 100 ①

17/6/2018

Dear Dr. Robson,

Re: Mr. Tom Riddle

DOB: 10/5/1999

I am writing to refer Mr. Riddle, a 20-year-old student, whose symptoms and signs are suggestive of asthma. Your expert assessment and further management would be highly appreciated.

Mr. Riddle, who is a non-smoker, has a past history of asthma related to his father. He has had asthma since 1996. He has experienced attacks of eczema. However, no allergy has been noticed.

On 11/6/2018, Mr. Tom attended my clinic with the complaints of a 2-week duration of breathlessness and persistent coughing associated with wheezing and eye itching. These manifestations may be precipitated by a contact with cats and dusty carpet or sleeping on floor, and were severe enough to miss his classes. His chest x-ray was clear. The FEV₁ was 3.51 pre-bronchodilator and 4.35 post-bronchodilator. Consequently, oral prednisone, 5mg for 10 days, Albuterol inhaler and Symbicort were prescribed. He was advised on allergen management.

Two weeks later, Mr. Tom's condition has deteriorated. Despite increasing Albuterol usage up to 6 times per day, his asthma has flared up. Complicated with a sleep disturbance. His examination revealed tachycardia, tachypnea and high blood pressure. Expiratory wheeze had diminished. Ling. Swells on both sides have been detected upon his chest examination. The arterial blood gases test has revealed hypoxia, normocapnia and oxygen saturation of 88%. Based on that, 3 litres of oxygen inhalation, Albuterol and I.V. corticosteroid were given. A discussion regarding avoidance of environmental triggers and technique of proper inhalation was conducted.

In view of above, kindly evaluate his condition, and counsel him.

OET Writing sub-test - Answer booklet 2 on environmental management.

Should you have any queries, please do not hesitate to contact me.

Yours Sincerely,
Doctor

